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tected interest,” that belongs to them. *Spokeo, supra*, at 339 (internal quotation marks omitted). Thus, under a proper understanding of Article III, plaintiffs lack standing and, consequently, this Court lacks jurisdiction.

II

Even if the plaintiffs had standing, the Court would still lack the authority to enjoin Louisiana’s law, which represents a constitutionally valid exercise of the State’s traditional police powers. The plurality and THE CHIEF JUSTICE claim that the Court’s judgment is dictated by “our precedents,” particularly *Whole Woman’s Health. Ante*, at 342 (plurality opinion); see also *ante*, at 344–345, 354–358 (ROBERTS, C. J., concurring in judgment). For the detailed reasons explained by JUSTICE ALITO, this is not true. *Post*, at 379–399 (dissenting opinion).

But today’s decision is wrong for a far simpler reason: The Constitution does not constrain the States’ ability to regulate or even prohibit abortion. This Court created the right to abortion based on an amorphous, unwritten right to privacy, which it grounded in the “legal fiction” of substantive due process, *McDonald v. Chicago*, 561 U. S. 742, 811 (2010) (THOMAS, J., concurring in part and concurring in judgment). As the origins of this jurisprudence readily demonstrate, the putative right to abortion is a creation that should be undone.

A

The Court first conceived a free-floating constitutional right to privacy in *Griswold v. Connecticut*, 381 U. S. 479 (1965). In that case, the Court declared unconstitutional a state law prohibiting the use of contraceptives, finding that it violated a married couple’s “right of privacy.” *Id.*, at 486. The Court explained that this right could be found in the “penumbras” of *five* different Amendments to the Constitution—the First, Third, Fourth, Fifth, and Ninth. *Id.*, at 484. Rather than explain what free speech or the quartering of

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troops had to do with contraception, the Court simply declared that these rights had created “zones of privacy” with their “penumbras,” which were “formed by emanations from those guarantees that help give them life and substance.” *Ibid.* This reasoning is as mystifying as it is baseless.

As Justice Black observed in his dissent, this general “right of privacy” was never before considered a constitutional guarantee protecting citizens from governmental intrusion. *Id.*, at 508–510. Rather, the concept was one of tort law, championed by Samuel Warren and the future Justice Louis Brandeis in their 1890 Harvard Law Review article entitled, “The Right to Privacy.” 4 Harv. L. Rev. 193. Over 20 years after the Fourteenth Amendment was ratified and a century after the Bill of Rights was adopted, Warren and Brandeis were among the first to advocate for this privacy right in the context of tort relief for those whose personal information and private affairs were exploited by others. *Id.*, at 193, 195–196, 214–220. By “exalting a phrase . . . used in discussing grounds for tort relief, to the level of a constitutional rule,” the Court arrogated to itself the “power to invalidate any legislative act which [it] find[s] irrational, unreasonable[,] or offensive” as an impermissible “interfere[nce] with ‘privacy.’” *Griswold, supra*, at 510, n. 1, 511 (Black, J., dissenting).

Just eight years later, the Court utilized its newfound power in *Roe v. Wade*, 410 U.S. 113 (1973). There, the Court struck down a Texas law restricting abortion as a violation of a woman’s constitutional “right of privacy,” which it grounded in the “concept of personal liberty” purportedly protected by the Due Process Clause of the Fourteenth Amendment. *Id.*, at 153. The Court began its legal analysis by openly acknowledging that the Constitution’s text does not “mention any right of privacy.” *Id.*, at 152. The Court nevertheless concluded that it need not bother with our founding document’s text, because the Court’s prior decisions—chief among them *Griswold*—had already divined

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such a right from constitutional penumbras. *Roe*, 410 U. S., at 152. Without any legal explanation, the Court simply concluded that this unwritten right to privacy was “broad enough to encompass a woman’s [abortion] decision.” *Id.*, at 153.

B

Roe is grievously wrong for many reasons, but the most fundamental is that its core holding—that the Constitution protects a woman’s right to abort her unborn child—finds no support in the text of the Fourteenth Amendment. *Roe* suggests that the Due Process Clause’s reference to “liberty” could provide a textual basis for its novel privacy right. *Ibid.* But that Clause does not guarantee liberty *qua* liberty. Rather, it expressly contemplates the *deprivation* of liberty and requires only that such deprivations occur through “due process of law.” Amdt. 14, § 1. As I have previously explained, there is “‘considerable historical evidence support[ing] the position that “due process of law” was [originally understood as] a separation-of-powers concept . . . forbidding only deprivations not authorized by legislation or common law.’” *Johnson v. United States*, 576 U. S. 591, 623 (2015) (opinion concurring in judgment) (quoting D. Currie, *The Constitution in the Supreme Court: The First Hundred Years 1789–1888*, p. 272 (1985)). Others claim that the original understanding of this Clause requires that “statutes that purported to empower the other branches to deprive persons of rights without adequate procedural guarantees [be] subject to judicial review.” Chapman & McConnell, *Due Process as Separation of Powers*, 121 *Yale L. J.* 1672, 1679 (2012). But, whatever the precise requirements of the Due Process Clause, “[t]he notion that a constitutional provision that guarantees only ‘process’ before a person is deprived of life, liberty, or property could define the substance of those rights strains credulity for even the most casual user of words.” *McDonald*, 561 U. S., at 811 (opinion of THOMAS, J.).

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More specifically, the idea that the Framers of the Fourteenth Amendment understood the Due Process Clause to protect a right to abortion is farcical. See *Roe*, 410 U. S., at 174–175 (Rehnquist, J., dissenting). In 1868, when the Fourteenth Amendment was ratified, a majority of the States and numerous Territories had laws on the books that limited (and in many cases nearly prohibited) abortion. See *id.*, at 175, n. 1.⁷ It would no doubt shock the public at that time to learn that one of the new constitutional Amendments contained hidden within the interstices of its text a right to abortion. The fact that it took this Court over a century to find that right all but proves that it was more than hidden—it simply was not (and is not) there.

C

Despite the readily apparent illegitimacy of *Roe*, “the Court has doggedly adhered to [its core holding] again and

⁷See, e.g., Ala. Rev. Code § 3605 (1867); Terr. of Ariz., Howell Code, ch. 10, § 45 (1865); Ark. Rev. Stat., ch. 44, div. III, Art. II, § 6 (1838); 1861 Cal. Stat., ch. 521, § 45, p. 588; Colo. (Terr.) Rev. Stat. § 42 (1868); Conn. Gen. Stat., Tit. 12, §§ 22–24 (1861); Fla. Acts 1st Sess., ch. 1637, subch. III, §§ 10, 11, ch. 8, §§ 9, 10 (1868); Terr. of Idaho Laws, Crimes and Punishments § 42 (1864); Ill. Stat., ch. 30, § 47 (1868); Ind. Laws ch. LXXXI, § 2 (1859); Iowa Rev. Gen. Stat., ch. 165, § 4221 (1860); Kan. Gen. Stat., ch. 31, §§ 14, 15, 44 (1868); La. Rev. Stat., Crimes and Offenses § 24 (1856); Me. Rev. Stat., Tit. XI, ch. 124, § 8 (1857); 1868 Md. Laws ch. 179, § 2, p. 315; Mass. Gen. Stat., ch. 165, § 9 (1860); Mich. Rev. Stat., Tit. XXX, ch. 153, §§ 32, 33, 34 (1846); Terr. of Minn. Rev. Stat., ch. 100, §§ 10, 11 (1851); Miss. Rev. Code, ch. LXIV, Arts. 172, 173 (1857); Mo. Rev. Stat., Art. II, §§ 9, 10, 36 (1835); Terr. of Mont. Laws, Criminal Practice Acts § 41 (1864); Terr. of Neb. Rev. Stat., Crim. Code § 42 (1866); Terr. of Nev. Laws ch. 28, § 42 (1861); 1848 N. H. Laws ch. 743, §§ 1, 2, p. 708; 1849 N. J. Laws pp. 266–267; 1854 Terr. of N. M. Laws ch. 3, §§ 10, 11, p. 88; 1846 N. Y. Laws ch. 22, § 1, p. 19; 1867 Ohio Laws § 2, pp. 135–136; Ore. Gen. Laws, Crim. Code, ch. XLIII, § 509 (1845–1864); 1860 Pa. Laws no. 374, §§ 87, 88, 89, pp. 404–405; Tex. Gen. Stat. Dig., Penal Code, ch. VII, Arts. 531–536 (1859); 1867 Vt. Acts & Resolves no. 57, §§ 1, 3, pp. 64–66; 1848 Va. Acts, Tit. II, ch. 3, § 9, p. 96; Terr. of Wash. Stat., ch. II, §§ 37, 38 (1854); Wis. Rev. Stat., ch. 164, §§ 10, 11, ch. 169, §§ 58, 59 (1858).

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again, often to disastrous ends.” *Gamble v. United States*, 587 U. S. 678, 725 (2019) (THOMAS, J., concurring). In doing so, the Court has repeatedly invoked *stare decisis*. See, e. g., *Casey*, 505 U. S., at 854–869. And today, a majority of the Court insists that this doctrine compels its result. See *ante*, at 344 (plurality opinion); *ante*, at 344–345, 354 (opinion of ROBERTS, C. J.).

The Court’s current “formulation of the *stare decisis* standard does not comport with our judicial duty under Article III,” which requires us to faithfully interpret the Constitution. *Gamble*, 587 U. S., at 711 (THOMAS, J., concurring). Rather, when our prior decisions clearly conflict with the text of the Constitution, we are required to “privilege [the] text over our own precedents.” *Id.*, at 719. Because *Roe* and its progeny are premised on a “demonstrably erroneous interpretation of the Constitution,” we should not apply them here. 587 U. S., at 719 (THOMAS, J., concurring).

Even under THE CHIEF JUSTICE’s approach to *stare decisis*, continued adherence to these precedents cannot be justified. *Stare decisis* is “not an inexorable command,” *ante*, at 346 (internal quotation marks omitted), and this Court has recently overruled a number of poorly reasoned precedents that have proved themselves to be unworkable, see *Knick v. Township of Scott*, 588 U. S. 180, 202–205 (2019) (ROBERTS, C. J., for the Court); *Franchise Tax Bd. of Cal. v. Hyatt*, 587 U. S. 230, 248–249 (2019); *Janus v. State, County, and Municipal Employees*, 585 U. S. 878, 916–929 (2018). As I have already demonstrated, *supra*, at 371–374, *Roe*’s reasoning is utterly deficient—in fact, not a single Justice today attempts to defend it.

Moreover, the fact that no five Justices can agree on the proper interpretation of our precedents today evinces that our abortion jurisprudence remains in a state of utter entropy. Since the Court decided *Roe*, Members of this Court have decried the unworkability of our abortion case law and repeatedly called for course corrections of varying degrees.

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See, e. g., 410 U. S., at 171–178 (Rehnquist, J., dissenting); *Doe v. Bolton*, 410 U. S. 179, 221–223 (1973) (White, J., dissenting); *Akron v. Akron Center for Reproductive Health, Inc.*, 462 U. S. 416, 452–466 (1983) (O'Connor, J., dissenting); *Thornburgh v. American College of Obstetricians and Gynecologists*, 476 U. S. 747, 785–797 (1986) (White, J., dissenting); *Webster v. Reproductive Health Services*, 492 U. S. 490, 532–537 (1989) (Scalia, J., concurring in part and concurring in judgment); *Casey*, 505 U. S., at 944–966 (Rehnquist, C. J., concurring in judgment in part and dissenting in part); *id.*, at 979–1002 (Scalia, J., concurring in judgment in part and dissenting in part); *Stenberg*, 530 U. S., at 953–956 (Scalia, J., dissenting); *id.*, at 980–983 (THOMAS, J., dissenting); *Whole Woman's Health*, 579 U. S., at 633–638 (THOMAS, J., dissenting). In *Casey*, the majority claimed to clarify this “jurisprudence of doubt,” 505 U. S., at 844, but our decisions in the decades since then have only demonstrated the folly of that assertion, see *Stenberg*, 530 U. S., at 953–956 (Scalia, J., dissenting); *id.*, at 960–979 (Kennedy, J., dissenting); *Whole Woman's Health*, *supra*, at 633–638 (THOMAS, J., dissenting). They serve as further evidence that this Court's abortion jurisprudence has failed to deliver the “‘principled and intelligible’” development of the law that *stare decisis* purports to secure. *Ante*, at 346 (opinion of ROBERTS, C. J.) (quoting *Vasquez v. Hillery*, 474 U. S. 254, 265 (1986)).

THE CHIEF JUSTICE advocates for a Burkean approach to the law that favors adherence to “‘the general bank and capital of nations and of ages.’” *Ante*, at 345 (quoting 3 E. Burke, *Reflections on the Revolution in France* 110 (1790)). But such adherence to precedent was conspicuously absent when the Court broke new ground with its decisions in *Griswold* and *Roe*. And no one could seriously claim that these revolutionary decisions—or *Whole Woman's Health*, decided just four Terms ago—are part of the “*inheritance from our forefathers*,” fidelity to which demonstrates “reverence to antiquity.” E. Burke, *Reflections on the Revolution in France* 27–28 (J. Pocock ed. 1987).

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More importantly, we exceed our constitutional authority whenever we “appl[y] demonstrably erroneous precedent instead of the relevant law’s text.” *Gamble, supra*, at 711–712 (THOMAS, J., concurring). Because we can reconcile neither *Roe* nor its progeny with the text of our Constitution, those decisions should be overruled.

* * *

Because we lack jurisdiction and our abortion jurisprudence finds no basis in the Constitution, I respectfully dissent.⁸

JUSTICE ALITO, with whom JUSTICE GORSUCH joins, with whom JUSTICE THOMAS joins except as to Parts III–C and IV–F, and with whom JUSTICE KAVANAUGH joins as to Parts I, II, and III, dissenting.

The majority bills today’s decision as a facsimile of *Whole Woman’s Health v. Hellerstedt*, 579 U. S. 582 (2016), and it’s true they have something in common. In both, the abortion right recognized in this Court’s decisions is used like a bulldozer to flatten legal rules that stand in the way.

In *Whole Woman’s Health*, *res judicata* and our standard approach to severability were laid low. Even *Planned Parenthood of Southeastern Pa. v. Casey*, 505 U. S. 833 (1992), was altered.

Today’s decision claims new victims. The divided majority cannot agree on what the abortion right requires, but it nevertheless strikes down a Louisiana law, Act 620, that the legislature enacted for the asserted purpose of protecting

⁸I agree with JUSTICE ALITO’s application of our precedents except in Part IV–F of his opinion, but I would not remand for further proceedings. Because plaintiffs lack standing under Article III, I would instead remand with instructions to dismiss for lack of jurisdiction. Alternatively, if I were to reach the merits because a majority of the Court concludes we have jurisdiction, I would affirm, as plaintiffs have failed to carry their burden of demonstrating that Act 620 is unconstitutional, even under our precedents.

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women's health. To achieve this end, the majority misuses the doctrine of *stare decisis*, invokes an inapplicable standard of appellate review, and distorts the record.

The plurality eschews the constitutional test set out in *Casey* and instead employs the balancing test adopted in *Whole Woman's Health*. The plurality concludes that the Louisiana law does nothing to protect the health of women, but that is disproved by substantial evidence in the record. And the plurality upholds the District Court's finding that the Louisiana law would cause a drastic reduction in the number of abortion providers in the State even though this finding was based on an erroneous legal standard and a thoroughly inadequate factual inquiry.

THE CHIEF JUSTICE stresses the importance of *stare decisis* and thinks that precedent, namely, *Whole Woman's Health*, dooms the Louisiana law. But at the same time, he votes to overrule *Whole Woman's Health* insofar as it changed the *Casey* test.

Both the plurality and THE CHIEF JUSTICE hold that abortion providers can invoke a woman's abortion right when they attack state laws that are enacted to protect a woman's health. Neither waiver nor *stare decisis* can justify this holding, which clashes with our general rule on third-party standing. And the idea that a regulated party can invoke the right of a third party for the purpose of attacking legislation enacted to protect the third party is stunning. Given the apparent conflict of interest, that concept would be rejected out of hand in a case not involving abortion.

For these reasons, I cannot join the decision of the Court. I would remand the case to the District Court and instruct that court, before proceeding any further, to require the joinder of a plaintiff with standing. If a proper plaintiff is added, the District Court should conduct a new trial and determine, based on proper evidence, whether enforcement of Act 620 would diminish the number of abortion providers in the State to such a degree that women's access to abor-

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tions would be substantially impaired. In making that determination, the court should jettison the nebulous “good faith” test that it used in judging whether the physicians who currently lack admitting privileges would be able to obtain privileges and thus continue to perform abortions if Act 620 were permitted to take effect. Because the doctors in question (many of whom are or were plaintiffs in this case) stand to lose, not gain, by obtaining privileges, the court should require the plaintiffs to show that these doctors sought admitting privileges with the degree of effort that they would expend if their personal interests were at stake.

I

Under our precedent, the critical question in this case is whether the challenged Louisiana law places a “substantial obstacle in the path of a woman seeking an abortion of a nonviable fetus.” *Casey*, 505 U. S., at 877 (plurality opinion). If a law like that at issue here does not have that effect, it is constitutional. *Id.*, at 884 (joint opinion of O’Connor, Kennedy, and Souter, JJ.).

Petitioners urge us to adopt a rule that is more favorable to abortion providers. At oral argument, their attorney maintained that a law that has no effect on women’s access to abortion is nevertheless unconstitutional if it is not needed to protect women’s health. See Tr. of Oral Arg. 18–19. Of course, that is precisely the argument one would expect from a business that wishes to be free from burdensome regulations. But unless an abortion law has an adverse effect *on women*, there is no reason why the law should face greater constitutional scrutiny than any other measure that burdens a regulated entity in the name of health or safety. See *Casey*, 505 U. S., at 884–885 (joint opinion). Many state and local laws that are justified as safety measures rest on debatable empirical grounds. But when a party saddled with such restrictions challenges them as a violation of due process, our cases call for the restrictions to be sustained if

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“it might be thought that the particular legislative measure was a rational way” to serve a valid interest. See *Williamson v. Lee Optical of Okla., Inc.*, 348 U.S. 483, 488 (1955). The test that petitioners advocate would give abortion providers an unjustifiable advantage over all other regulated parties, and for that reason, it was rejected in *Casey*. See 505 U.S., at 851 (majority opinion).

Casey also rules out the balancing test adopted in *Whole Woman’s Health*. *Whole Woman’s Health* simply misinterpreted *Casey*, and I agree that *Whole Woman’s Health* should be overruled insofar as it changed the *Casey* test. Unless *Casey* is reexamined—and Louisiana has not asked us to do that—the test it adopted should remain the governing standard.

II

Because the plurality adheres to the balancing test adopted in *Whole Woman’s Health*, it considers whether the Louisiana law helps to protect the health of women seeking abortions, and it concludes that “nothing in the record indicates that the background vetting for admitting privileges adds significantly to the vetting that the State Board of Medical Examiners already provides.” *Ante*, at 380. THE CHIEF JUSTICE seems to agree, *ante*, at 356–357 (opinion concurring in judgment), although it is unclear why this issue matters under the test he favors.

In any event, contrary to the view taken by the plurality and (seemingly) by THE CHIEF JUSTICE, there is ample evidence in the record showing that admitting privileges help to protect the health of women by ensuring that physicians who perform abortions meet a higher standard of competence than is shown by the mere possession of a license to practice. In deciding whether to grant admitting privileges, hospitals typically undertake a rigorous investigative process to ensure that a doctor is responsible and competent and has the training and experience needed to perform the procedures for which the privileges are sought. As the Fifth Cir-

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cuit explained, “hospitals verify an applicant’s surgical ability, training, education, experience, practice record, and criminal history. These factors are reviewed by a board of multiple physicians.” *June Medical Services, L. L. C. v. Gee*, 905 F. 3d 787, 805, n. 53 (2018).

The standards used by the great majority of hospitals in deciding whether to grant privileges clearly show that hospitals demand proof of a higher level of competence. The Joint Commission, a nonprofit organization that accredits healthcare institutions, has issued standards for granting admitting privileges, and all of the hospitals whose rules are relevant here (and the vast majority of Louisiana hospitals) comply with those standards.¹ These standards call for an examination of each applicant’s licensure, education, training, and current competence. See Joint Commission, 2020 Hospital Accreditation Standards, pp. MS–23, 25, 26, 29. They require an examination of a doctor’s health records, clinical data on performance, and peer recommendations, and they demand that a hospital make a careful assessment of the procedures a physician may perform. *Ibid.*

Dr. Robert Marier, the former director of the Louisiana Board of Medical Examiners (and the former dean of Louisiana State University Medical School), testified that the process conducted by hospitals in deciding whether to grant admitting privileges is “the primary way of determining competency.” App. 818. That process, he explained, “thoroughly vet[s] the qualifications of [applicants] to ensure that [they] are competent to provide the services that are in question.” *Ibid.*

June Medical’s expert, Dr. Eva Pressman, agreed that “admitting privileges can serve the function of providing an

¹ Quality Check, Find a Gold Seal Health Care Organization (2020), <https://www.qualitycheck.org/search/?keyword=louisiana#keyword=louisiana&accreditationprogram=Hospital> (listing “[o]rganizations that have achieved The Gold Seal of Approval from the Joint Commission”).

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evaluation mechanism for physician competency.” *Id.*, at 1042, 1091; Record 10864. Doe 3, one of the doctors who currently performs abortions in Louisiana, also acknowledged the credentialing value of admitting privileges, App. 247–248, as did Doe 4, another Louisiana abortion doctor, Record 14155.

Although the plurality contends that the review conducted by hospitals adds little to the vetting undertaken by the State Board of Medical Examiners (Board), that is not true. Hospitals look beyond the mere possession of a license, and they do that for very obvious reasons. If nothing else, their review process serves the hospitals’ interests by diminishing the risk of awards for malpractice committed by doctors practicing on their premises. In Louisiana, hospitals that perform negligent credentialing cannot benefit from the State’s medical malpractice cap. See *Billeaudeau v. Opelousas General Hospital Auth.*, 2016–0846, p. 21 (La. 10/19/16), 218 So. 3d 513, 527. In addition, a hospital’s “Medicare participation and other certifications depend on completing the credentialing process.”²

The review conducted by hospitals goes beyond that of the Board in another way: it is continuous. Under the Joint Commission Standards, hospitals must monitor physicians with admitting privileges and can therefore make a running assessment of their competence. See Record 11850. The Board, on the other hand, conducts an inquiry before initially issuing a license, but the annual license renewal process entails nothing more than completing a standard form, paying the required fee, and documenting a certain number of con-

²Ryan, *Negligent Credentialing: A Cause of Action for Hospital Peer Review Decisions*, 59 *How. L. J.* 413, 419 (2016); see also Eskine, *Square Pegs and Round Holes: Antitrust Law and the Privileging Decision*, 44 *U. Kan. L. Rev.* 399, 401 (1996) (“[H]ospitals have strong incentives to award staff privileges only to those physicians who have proven to be capable and knowledgeable professionals”).

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tinuing medical education credits. See 46 La. Admin. Code, pt. XLV, § 417 (2020).

Because hospitals continue to evaluate doctors after privileges are granted, they may discover information that assists the Board in carrying out its responsibilities. In the past, hospitals have forwarded such information to the Board, and such referrals have led the Board to take serious disciplinary actions.³

The record shows that the vetting conducted by hospitals goes far beyond what is done at Louisiana abortion clinics. Some clinics demand nothing more than possession of a license. Take the example of petitioner June Medical. Doe 3, the only person at that clinic who evaluates applicants, testified that he does not perform background checks of any kind, not even criminal records checks. App. 249–250. In the past, Doe 3 hired a radiologist and ophthalmologist to perform abortions. *Id.*, at 249.

Delta Clinic in Baton Rouge and Women’s Clinic in New Orleans have similarly lax practices. Leroy Brinkley, the president of both clinics, testified before a Pennsylvania grand jury that, in making hiring decisions, “I don’t judge the license. If they have a license and the state gave the license, it’s not for me to determine if they are capable.”⁴ A “‘background check,’” he said, is not within his “‘framework.’”⁵

Doe 4, who practiced at the now-defunct Causeway Clinic near New Orleans, recounted the meager vetting that occurred when he was hired at that facility. He had to produce a valid medical license and DEA license but was

³ Brief for 207 Members of Congress as *Amici Curiae* 18–19 (lifetime ban from obstetric surgery in Louisiana); *id.*, at 19–20 (one-year probation of medical license).

⁴ Brief for Louisiana State Legislators as *Amici Curiae* 8–9; App. to *id.*, at 67a.

⁵ *Ibid.*

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not required “to undergo anything similar to review by a credentials committee.” Record 14156.

In light of these practices, it is no surprise that the Louisiana Department of Health has issued Statements of Deficiency against abortion facilities for failing to adopt “‘a detailed credentialing process for physicians,’” failing to investigate “‘possible restrictions’” on physicians’ licenses, and failing to look into “‘evidence of prior malpractice claims/settlements.’”⁶

Louisiana adopted Act 620 in the aftermath of the Kermit Gosnell grand jury report, which expounded on the failures of regulatory oversight that allowed Gosnell’s practices to continue for an extended period. See Report of Grand Jury in No. 0009901–2008 (1st Jud. Dist. Pa., Jan. 14, 2011). The grand jury concluded that closer supervision would have uncovered Gosnell’s egregious health and safety violations. Gosnell had a medical license, but it is doubtful that any hospital would have given him admitting privileges.

In sum, contrary to the plurality’s assertion, there is ample evidence in the record showing that requiring admitting privileges has health and safety benefits. There is certainly room for debate about the need for this requirement, but under our case law, this Court’s task is not to ascertain whether a law “adds significantly” to the existing regulatory framework. Instead, when confronted with a genuine dispute about a law’s benefits, we have afforded legislatures “wide discretion” in assessing whether a regulation serves a legitimate medical need and is medically reasonable even in the face of medical and scientific uncertainty. *Gonzales v. Carhart*, 550 U. S. 124, 163 (2007); *Mazurek v. Armstrong*, 520 U. S. 968, 973 (1997) (*per curiam*); *Akron v. Akron Center for Reproductive Health, Inc.*, 462 U. S. 416, 458 (1983) (O’Connor, J., dissenting) (“[L]egislatures are better suited” than courts “to make the necessary factual judgments in this

⁶ *Id.*, at 9.

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area”); accord, *Barsky v. Board of Regents of Univ. of N. Y.*, 347 U. S. 442, 451 (1954) (State has “legitimate concern for maintaining high standards of professional conduct” in the practice of medicine). Louisiana easily satisfied this standard.

For these reasons, both the plurality and THE CHIEF JUSTICE err in concluding that the admitting-privileges requirement serves no valid purpose.

III

They also err in their assessment of Act 620’s likely effect on access to abortion. They misuse the doctrine of *stare decisis* and the standard of appellate review for findings of fact.

A

Stare decisis is a major theme in the plurality opinion and that of THE CHIEF JUSTICE. Both opinions try to create the impression that this case is the same as *Whole Woman’s Health* and that *stare decisis* therefore commands the same result. In truth, however, the two cases are very different. While it is certainly true that the Texas and Louisiana *statutes* are largely the same, the two cases are not. The decision in *Whole Woman’s Health* was not based on the face of the Texas statute, but on an empirical question, namely, the effect of the statute on access to abortion in that State. 579 U. S., at 612. The Court’s answer to that question depended on numerous factors that may differ from State to State, including the demand for abortions, the number and location of abortion clinics and physicians, the geography of the State, the distribution of the population, and the ability of physicians to obtain admitting privileges. *Id.*, at 612–614. There is no reason to think that a law requiring admitting privileges will necessarily have the same effect in every State. As a result, just because the Texas admitting-privileges requirement was found by this Court, based on evidence in the record of that case, to have substantially re-

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duced access to abortion in that State, it does not follow that Act 620 would have comparable effects in Louisiana. See *id.*, at 610–614 (reviewing Texas record). The two States are neighbors, but they are not the same. Accordingly, the record-based empirical determination in *Whole Woman’s Health* is not controlling here.

The suggestion that *Whole Woman’s Health* is materially identical to this case is ironic, since the two cases differ in a way that was critical to the Court’s reasoning in *Whole Woman’s Health*, *i. e.*, the difference between a pre-enforcement facial challenge and a post-enforcement challenge based on evidence of the law’s effects. See *id.*, at 599. Before the Texas law went into effect, abortion providers mounted an unsuccessful facial challenge, arguing that the law would drastically limit abortion access. The Fifth Circuit held that the plaintiffs had not shown that the law would create a substantial obstacle for women seeking abortions, and a final judgment was entered against them. *Planned Parenthood of Greater Tex. Surgical Health Servs. v. Abbott*, 748 F. 3d 583, 590, 605 (2014). Then, after the law had been in operation for some time, many of the same plaintiffs filed a second suit and again argued that the admitting-privileges requirement violated *Casey*. *Whole Woman’s Health v. Cole*, 790 F. 3d 563, 577, and n. 14 (CA5 2015). The state defendants sought dismissal based on the doctrine of claim preclusion, but the *Whole Woman’s Health* majority rejected that argument. 579 U. S., at 599.

Why? Two words: “changed circumstances.” *Id.*, at 601. According to the Court, the pre-enforcement facial challenge was not the same “claim” as the post-enforcement claim because the “postenforcement consequences” of the challenged Texas law were “*unknowable* before [the law] went into effect.” *Id.*, at 602 (emphasis added); see also *ibid.* (“[I]t was still unclear how many clinics would be affected”); *id.*, at 599 (discussing “new material facts”); *id.*, at 602 (recounting “later, concrete factual developments”).

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The present case is in the same posture as the pre-enforcement facial challenge to the Texas law, and it should therefore be obvious that this Court's decision in *Whole Woman's Health* is not controlling.

B

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Aside from suggesting that *Whole Woman's Health* is dispositive, the plurality and THE CHIEF JUSTICE provide one other reason for concluding that Act 620, if allowed to go into effect, would create a substantial obstacle for women seeking abortions. Pointing to the District Court's finding that the Louisiana law would have a drastic effect on abortion access, *June Medical Services, LLC v. Kliebert*, 250 F. Supp. 3d 27, 87 (MD La. 2017), the plurality and THE CHIEF JUSTICE note that findings of fact may be overturned only if clearly erroneous, and they see no such error here. *Ante*, at 322 (opinion of BREYER, J.); *ante*, at 358 (opinion of ROBERTS, C. J.). In taking this approach, they overlook the flawed legal standard on which the District Court's finding depends, and they ignore the gross deficiencies of the evidence in the record.

Because the Louisiana law was not allowed to go into effect for any appreciable time, it was necessary for the District Court to predict what its effects would be. Attempting to do that, the court apparently concluded that none of the doctors who currently perform abortions in the State would be replaced if the admitting-privileges requirement forced them to leave abortion practice. 250 F. Supp. 3d, at 82. That inference is debatable, as it primarily rests on the anecdotal testimony of June Medical's administrator. See *id.*, at 81–82; App. 113–114. Neither the plurality nor THE CHIEF JUSTICE explains why it should be accepted. That alone casts doubt on the finding to which the majority defers, but the problems with the finding do not stop there.

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The finding was based on a fundamentally flawed test. In attempting to ascertain how many of the doctors who perform abortions in the State would have to leave abortion practice for lack of admitting privileges, the District Court received evidence in a variety of forms—some live testimony, but also deposition transcripts, declarations, and even letters from counsel—about the doctors’ unsuccessful efforts to obtain privileges. The District Court considered whether these doctors had proceeded in “good faith”; it found that they all met that standard; and it therefore concluded that the law would leave the State with very few abortion providers.

2

Under the reasoning just described, the factual finding on which the plurality and THE CHIEF JUSTICE rely—that the Louisiana law would drastically reduce access to abortion in the State—depends on the District Court’s finding that the doctors in question exercised “good faith” in their quest for privileges, but that test is woefully deficient.

It has aptly been said that “good faith” “‘is an elusive idea, taking on different meanings and emphases as we move from one context to another.’” Black’s Law Dictionary 836 (11th ed. 2019). What the District Court understood the term to mean in the present context is uncertain, but this is clear: The District Court ignored a factor of the utmost importance, the incentives of the doctors in question.

When the District Court made its assessment of the doctors’ “good faith,” enforcement of Act 620 had been preliminarily enjoined, and the doctors surely knew that enforcement would be permanently barred if the lawsuit was successful. Thus, the doctors had everything to lose and nothing to gain by obtaining privileges.⁷ Two of the doc-

⁷ Petitioners maintain that an unsuccessful admitting-privileges application is a “stain” on a doctor’s medical record, because the rejection could appear in a federal database and would need to be disclosed on future applications for admitting privileges. Brief for Petitioners in No. 18–

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tors—Does 1 and 2—are petitioners and cross-respondents in this Court. Two others, Does 5 and 6, were plaintiffs earlier but dropped out for unexplained reasons. See App. 1327. And Doe 3, although not a plaintiff, is the medical director of June Medical, a party to this case. *Id.*, at 186, 206, 245.

If these doctors had secured privileges, that would have tended to defeat the lawsuit. Not only that, acquiring privileges would have subjected all the doctors to the previously described hospital monitoring, as well as any other obligations that a hospital imposed on doctors with privileges, such as providing unpaid care for the indigent. See *infra*, at 396–397. Thus, in light of the situation at the time when the doctors made their attempts to get privileges, they had an incentive to do as little as they thought the District Court would demand, not as much as they would if they stood to benefit from success.

Given this incentive structure, the District Court’s “good faith” test was not up to the task. Although the District Court did not define exactly what the test required, “good faith” might easily mean only that a doctor lacked the subjective intent to avoid getting privileges. See Black’s Law Dictionary, at 836 (defining “good faith” to mean, among other things, “absence of intent to defraud or seek unconscionable advantage”).

In light of the doctors’ incentives, more should have been required. The court should have asked whether the doctors’ efforts to acquire privileges were equal to the efforts they

1323, p. 41, n. 7. As the record in this case shows, there is reason to doubt that the prospect of rejection provides a sufficient incentive for doctors to pursue privileges vigorously. See *infra*, at 390–399. Perhaps that is because only rejections for lack of “professional competence or professional conduct” need to be disclosed to the relevant federal database. 45 CFR §§ 60.12, 60.3 (2019). Petitioners also have not explained how a non-competence-based rejection would have any bearing on future applications for privileges.

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would have made if they knew that their ability to continue to perform abortions was at stake. The District Court did not do that, and because its finding on abortion access rests on the wrong legal standard, it cannot stand. A finding based on an erroneous legal test is invalid; it cannot be sustained under the “clearly erroneous” rule. See *Abbott v. Perez*, 585 U.S. 379, 607 (2018) (“‘An appellate court has power to correct errors of law, including those that . . . infect . . . a finding of fact that is predicated on a misunderstanding of the governing rule of law’” (quoting *Bose Corp. v. Consumers Union of United States, Inc.*, 466 U.S. 485, 501 (1984))); *Pullman-Standard v. Swint*, 456 U.S. 273, 287 (1982) (similar); see also 9C C. Wright & A. Miller, *Federal Practice and Procedure* §2585, p. 392 (3d ed. 2008) (Wright & Miller) (“[I]t is axiomatic that the conclusions of law of the trial judge are not protected by the ‘clearly erroneous’ test”).⁸

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Not only did the District Court apply the wrong test, but the evidence in the record fails to show that the doctors made anything more than perfunctory efforts to obtain privileges.

There are three abortion clinics in Louisiana: June Medical, d/b/a Hope Clinic, in Shreveport; Delta Clinic in Baton Rouge; and Women’s Clinic in New Orleans. Five doctors perform abortions at those three locations: Doe 1, Doe 2, and Doe 3 at June Medical; Doe 5 at Delta Clinic and Women’s

⁸The plurality claims that my criticism of the District Court’s “good faith” standard “is not a legal argument,” and instead reflects a view of the facts—namely, that the Does acted in “bad faith.” *Ante*, at 328–329. But the District Court used “good faith” as the legal standard to assess whether Act 620 would cause the Does to stop performing abortions. Neither the District Court nor the plurality has defined “good faith.” Unless that term reflects what the doctors would have done if the incentives had been reversed—and the plurality does not argue that it does—there is a legal issue.